NOT QUITE A MIRACLE DRUG

As sale of the latest metabolic drugs surge due to their parallel weight-loss use, doctors warn that lasting health takes more than a fast drop on the scale

BY SONALI ACHARJEE | *Illustration by* NILANJAN DAS

AT 42, RAGHAVAN IYER, A CHENNAI-BASED LOGISTICS MANAGER, believed oral semaglutide—a prescription drug used to manage Type 2 diabetes and weight gain—would end his struggle with extra kilos. He began Rybelsus—a lower-dose, tablet version of the same drug family as the injectable weight-loss blockbusters Ozempic and Wegovy—a year ago. The results were swift: nearly 20 kg lost in six months. Yet, his joints ached from rapid muscle loss and binge urges broke through. "The weight dropped," he says, "but the anxiety didn't."

Iyer is one of thousands of Indians who have taken advantage of the revolution in 'metabolic medicine'. If metabolism is the set of chemical reactions in the body through hormones that convert food into energy and enable bodily functions, such medicines, in a first, treat metabolic disorders—extreme weight gain, high blood pressure and high blood sugar that can spiral into Type 2 diabetes—by striking at the very root of metabolic function, at the level of hormones. In medical terms, Rybelsus, Ozempic, Wegovy and others are all variants of the Peptide (GLP-1) Receptor Agonist or Semaglutide, a drug that promotes the release of the GLP-1 hormone—which triggers the secretion of insulin and slows the digestion of food—from the intestine. There is also the Tirzepatide drug Mounjaro—an injectable, dual agonist medication that targets both GLP1- and GIP hormones—which is also used to manage Type-2 diabetes and excess weight.

However, even as these drugs are being marketed as life-changing and Indians gleefully pop 'magic pills' that makes their excess weight disappear, doctors are progressively getting worried about health risks. According to them, the phrase 'weight-loss drug' has rewritten expectations—suggesting that a complex, lifelong condition can be solved by swallowing a pill. Biology, they say, never works that simply. Sustained weight loss and well-being demand much more, says Dr Ramen Goel, consultant diabetes and bariatric surgeon at Wockhardt Hospitals, Mumbai.

THE PERILS OF WEIGHT-LOSS PILLS

The side-effects of indiscriminate usage of these drugs extend beyond the usual gastrointestinal ones—nausea, diarrhoea and constipation. More serious risks include pancreatitis, gall bladder disease (including gallstones), acute kidney injury and worsening diabetic retinopathy (affecting retina and sight). Wegovy's US label also flags monitoring for depression or suicidal thoughts.

Rapid weight reduction on GLP-1 drugs can also involve lean muscle loss, particularly when appetite suppression is not compensated with adequate intake of protein. In Mumbai, a 49-year-old chartered accountant who lost 22 kg in under eight months suffered persistent knee pain and dizziness. "Scans showed loss of muscle mass and early